DSM LAB · STUDY TOGETHER

The Unfolding Patient: role-play cards

Patients 49–60.

One person plays the patient; the other is the social worker doing an intake. **Print single-sided**, so the answer page never sits on the back of the patient card.

How to play

1. **Patient:** read the setting and your opening line aloud. Keep the rest of the card to yourself.
2. **Social worker:** interview. Ask about anything: onset and how long, sleep, substances and medications, medical history, safety, family, work or school, culture.
3. **Patient:** answer only what is asked, in your own words, using the "If asked about" table. Don't volunteer the items under "Only if asked".
4. **Social worker:** when ready (try 10 minutes), name a diagnosis and say what you'd do or check next.
5. Turn to the answer page together. Read the teaching point and the checkpoint question.

Each answer page follows its patient card. Keep it face down until the social worker commits to a diagnosis.

Scoring, if you want it

3 points for the right diagnosis, plus 1 for naming the rule-out (substance, medication or medical cause) before deciding, plus 1 for a safe, assess-first next step. Swap roles any time.

Playing it in the app

The same patients appear in DSM Lab → The Unfolding Patient, one clue at a time. If you play a card here first, the app version becomes a quick review.

These cases are original teaching material. Any resemblance to real people is coincidental.

Patient 49: The night-class electrician

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

Setting: Tomasz, 41, a union electrician who started an online associate degree eight months ago, at an outpatient telehealth visit. His wife joins him on camera, and his mother joins by phone for part of the visit.

> *"My wife made this appointment. I think I'm just too old for school. My brain doesn't hold anything anymore."*

If asked about…

When did this start?	School started about eight months ago, and that's when it fell apart. Work's been okay. Honestly, my wife says I've always been like this.
School	I open the assignment, read two lines, check my phone, go get a snack. Then it's midnight. I miss steps in the instructions. My instructor said I'm close to failing.
Home	I lose my keys and phone every day. I've left the stove on twice this month. The garage is full of half-done projects. My wife says I interrupt everybody.
Childhood and school as a kid	I was the class clown. Teachers said I was smart but lazy. I always had my homework done; I just never turned it in. My mom has the report cards.
Sleep	Seven hours, no problem. I had a sleep study because I snore, and it was normal.
Alcohol, drugs, medications	Two beers on Saturday. No weed, no pills. I've never taken anyone's Adderall, if that's what you're asking.
Medical history	Healthy. My doctor checked my thyroid five months ago. No head injuries, no seizures.
Mood and safety	I'm frustrated, not depressed. I still enjoy fishing with my son. No, I've never thought about killing myself.
Family history	My son got diagnosed with ADHD last year. When the doctor read the checklist out loud, I thought, that's me. My dad was the same way, never sat still.
Work	On a job site, I'm good. The foreman gives me a list, I'm moving, I'm using my hands. The paperwork side, I avoid. My foreman does it for me.
Culture and context	My parents came from Poland when I was a baby. Back then nobody we knew had heard of ADHD; my teachers just said 'try harder.' I did try. It never worked for long.

Only if asked (don’t volunteer)

- His wife has seen the same pattern for fifteen years, at home as well as at school
- His mother kept report cards from second grade on with teacher comments
- His son was diagnosed with ADHD last year
- Normal sleep study and thyroid tests

Patient 49: answer and debrief

DIAGNOSIS

ADHD

Problems that seemed to start with school made adjustment disorder the lead. Clues 5 and 6 flip it: his wife describes the same pattern at home for fifteen years, and his report cards show several symptoms in early grade school, well before age 12. He has at least 5 inattentive symptoms (the adult threshold) in two settings for far more than 6 months, with no substance, sleep, thyroid, mood or anxiety explanation. Exam lesson: adult ADHD needs evidence of symptoms before 12, best gathered from a parent or old school records, because adult memory of childhood is unreliable; new demands often unmask a lifelong pattern.

What had to be ruled out

- Adjustment disorder: the pattern is lifelong, not a new reaction to school
- Major depressive disorder: mood and interest are intact; sleep is normal
- Generalized anxiety disorder: no excessive worry
- Sleep or medical causes: sleep study and thyroid tests were normal
- Substance-related problems: two beers weekly, no drugs or medications

Checkpoint: What would you ask or check NEXT?

A. Begin short-term coaching on stress and time management for the new demands of school

B. Refer him to his doctor to start ADHD medication

C. Ask about substances, sleep and health, and ask to talk with people who know him at home and knew him as a child **(best)**

The open question is whether this began eight months ago or is lifelong, and that needs collateral about other settings and childhood, plus a check for substance, sleep or medical causes. A treats an adjustment problem before the onset question is answered. B jumps to treatment before any diagnosis is made.

The case in six clues (as the app reveals it)

1. I'm drowning in these online classes. I miss steps in the assignments, every paper is late, and last week my instructor warned me I'm about to fail.
2. On the job I've been fine for twenty years; my foreman hands me a checklist and I work with my hands. The trouble started about eight months ago, when I signed up for school on top of work.
3. I'm not down most days. I still love fishing with my son, and I sleep seven hours like a log. I'm not a worrier; I just can't make myself start the boring stuff, and once I do, every text and noise pulls me off it.
4. I have two beers on Saturdays, no weed, nothing else, and no medications. Five months ago my doctor checked my thyroid and did a sleep study for my snoring, and both were normal.
5. Wife: It's not just school. At home he loses his keys and phone every day, leaves the stove on, starts five projects and finishes none, and talks over everyone at dinner. He's been like this since I met him fifteen years ago; school just took away his workarounds.
6. Mother (by phone): I still have his report cards. From second grade on, every teacher wrote the same thing: bright, but daydreams, never finishes his work, loses his homework, can't stay in his seat.

What a skilled clinician weighs after each clue

1. Adjustment disorder, Major depressive disorder, Generalized anxiety disorder, ADHD
2. Adjustment disorder, ADHD, Generalized anxiety disorder
3. Adjustment disorder, ADHD
4. Adjustment disorder, ADHD
5. ADHD, Adjustment disorder
6. ADHD

Sources: DSM-5-TR (APA, 2022), Neurodevelopmental Disorders: Attention-Deficit/Hyperactivity Disorder; Kooij et al. (2019), updated European consensus statement on adult ADHD, European Psychiatry 56:14-34

Patient 50: The scripted lunch

Setting: Ingrid, 33, a veterinary technician, at an outpatient clinic for a second opinion. Her older sister came with her.

> *"I read an article about women being missed, and I recognized myself. But I've been wrong before, so I want someone to actually look."*

If asked about…

When did this start?	Always, I think. My sister remembers more of my childhood than I do. The exhaustion got worse when I moved from the kennel to the front desk two years ago.
Social life	I've wanted friends since I was little. I don't know how people go from 'nice to see you' to texting every day. Two friendships ended and I still don't know why.
Conversation	I can talk about parasites for an hour. People say I don't let them talk. I miss sarcasm. I practice facial expressions in the mirror.
Sensory	Clothing tags, fluorescent lights, the grocery store intercom. I wear earplugs. I can smell the cleaning chemicals at work from the parking lot.
Routines and interests	Same lunch every day: turkey, pear, crackers. If my route to work is closed, my whole day is ruined. As a kid it was horses, every breed. Now it's animal parasites.
Sleep	I sleep okay on weeknights. On weekends I sleep twelve hours to recover. It's not like the depression, when I couldn't sleep at all.
Alcohol, drugs, medications	I take sertraline. A glass of wine maybe once a month. No drugs. They make the lights and sounds worse.
Medical history	Healthy. Hearing tested fine as a kid. No seizures, no head injury.
Mood and safety	In college, in a bad stretch, I had thoughts of not wanting to be here. I never made a plan or tried anything. Right now, no. My mood is okay; I'm just exhausted.
Family history	My nephew was diagnosed with autism at four. My dad eats the same breakfast every day and has one friend. Mom had depression.
Work	I'm good with the animals and very accurate. Coworkers say I'm blunt. My manager wrote 'needs to work on rapport' on my review.
Culture and context	In our family, quiet girls were good girls. I was praised for being no trouble. My nephew was diagnosed right away. Nobody looked at me.

Only if asked (don’t volunteer)

- Childhood lining up, insistence on sameness and intense interests (from her sister)
- Sensory sensitivity to tags, lights and sounds
- Nephew diagnosed with autism; father has similar traits
- Past passive thoughts of not wanting to be here in college; none now; never a plan or attempt

Patient 50: answer and debrief

DIAGNOSIS

Autism spectrum disorder

Long-standing depression and social anxiety labels fit the surface. Clue 3 flips it: she does not fear judgment; she cannot read the social rules. She shows all three social-communication deficits DSM-5-TR requires (reciprocity: one-sided talk and missed jokes; nonverbal communication: eye contact and not seeing that others are bored; and understanding relationships), plus at least 2 restricted, repetitive patterns (sameness, intense interests, sensory sensitivity) dating to early childhood. Exam lesson: women without intellectual impairment are often identified late because learned strategies mask the signs; symptoms must still trace to early development, so collateral matters, and co-occurring depression or anxiety should be assessed, not assumed to explain everything.

What had to be ruled out

- Social anxiety disorder: no fear of negative evaluation; it would not explain the restricted, repetitive patterns
- Avoidant personality disorder: the core problem is not understanding social rules, not fear of rejection, and repetitive patterns date to early childhood
- Schizoid personality disorder: she wants close relationships
- Social (pragmatic) communication disorder: restricted, repetitive behaviors are present
- Intellectual developmental disorder: read at four, completed college
- Obsessive-compulsive disorder: lining up and routines bring comfort, not relief from intrusive fears

Checkpoint: What would you ask or check NEXT?

A. Start graded exposure for social anxiety, beginning with eating lunch in the break room

B. Ask what she feels in social situations, and about sensory issues, routines, interests and her childhood, with family as collateral **(best)**

C. Suggest that her prescriber try a fourth antidepressant

Years of treatment have not helped, so the formulation needs a fresh look: is this fear of judgment, or difficulty understanding social rules with other neurodevelopmental signs? A treats social anxiety before the nature of her difficulty is known. C repeats a strategy that has failed three times without reassessing the diagnosis.

The case in six clues (as the app reveals it)

1. I've been treated for depression and social anxiety since college: three medications, two therapists. By Friday I'm so drained that I spend the whole weekend in bed with the curtains shut.
2. Before every conversation at work I write a script. I copy how my coworkers laugh and when they nod. It works, but by the end of the day I'm wiped out from performing.
3. I'm not afraid people will judge me; I just don't know the rules. Coworkers say I'm rude because I talk about animal parasites and don't notice they're bored. I can't tell when someone is joking, and eye contact feels like touching a hot stove.
4. I've wanted friends my whole life. I've never understood how people go from coworkers to friends, and the two friendships I had ended without my knowing what I did wrong.
5. Sister: When we were little she lined up her horse figures by breed and color and screamed if I moved one. She still eats the same lunch every day, cuts the tags out of every shirt, and wears earplugs in the grocery store because of the lights and noise.
6. Sister: Teachers called her shy and gifted. She read at four, but she spent every recess walking the fence line alone and never once had a sleepover. She's been this way as long as I can remember.

What a skilled clinician weighs after each clue

1. Major depressive disorder, Social anxiety disorder, Avoidant personality disorder
2. Social anxiety disorder, Autism spectrum disorder, Avoidant personality disorder
3. Autism spectrum disorder, Social anxiety disorder, Avoidant personality disorder
4. Autism spectrum disorder, Avoidant personality disorder
5. Autism spectrum disorder, Obsessive-compulsive disorder
6. Autism spectrum disorder

Sources: DSM-5-TR (APA, 2022), Neurodevelopmental Disorders: Autism Spectrum Disorder; American Psychiatric Association (2022), DSM-5-TR fact sheet, Autism Spectrum Disorder

Patient 51: The morning stomachache

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

Setting: Zainab, 7, in second grade, seen by the school social worker. Her father and the school nurse are there with her.

> *"Father: The school keeps calling me to come get her. I leave work, and then she's fine in the car. I don't know what's real."*

If asked about…

When did this start?	About seven weeks ago. I had emergency surgery for my appendix and was in the hospital three days. She stayed with my sister. Since I came home, she won't let me out of her sight.
School mornings	Stomachache, crying at the car door, begging me to stay. Saturdays she eats pancakes and plays. No stomachache.
What she worries about	She asks if I'm going to die. She asks me to text the nurse when I get to work so she knows I'm safe.
Sleep	She starts in her bed but ends up in ours every night. Some nights she dreams she's lost at the mall and wakes up screaming.
Medical history and medications	The pediatrician examined her two weeks ago. Belly fine, bloodwork fine. No medicines. She's healthy.
Safety	She has never said she wants to hurt herself. Nobody has hurt her. She's just scared.
Family history	My wife had panic attacks in college. I'm a worrier myself.
School and friends	Her teacher says she's bright and the kids like her. She's missed nine days this term. She used to love it.
Away from parents	She cried at her cousin's sleepover until I came at ten at night. She won't go to her grandmother's without me now.
Culture and context	We're Nigerian. My mother in Lagos says just let her sleep with us, that's how we were raised. But Zainab chose her own bed at four and was proud of it. This is fear, not habit.

Only if asked (don’t volunteer)

- Onset about seven weeks ago, after his three-day hospital stay
- Her fear is about harm to her father, not about classmates
- Nightmares about getting lost; will not sleep apart
- Normal pediatric exam

Patient 51: answer and debrief

DIAGNOSIS

Separation anxiety disorder

Morning stomachaches and new quietness in class made social anxiety the lead. Clue 4 flips it: her fear is that harm will come to her father, not that peers will judge her. She shows many signs (physical complaints before separation, worry about harm to a parent, refusing to sleep apart, nightmares about separation, shadowing her father) for about seven weeks, more than the 4 weeks required in children. Exam lesson: a stressor such as a parent's hospital stay often sets it off, but when full criteria are met, diagnose separation anxiety disorder rather than adjustment disorder; CBT with gradual exposure and parent coaching aims to keep the child in school.

What had to be ruled out

- Medical cause of stomachaches: normal pediatric exam; pain follows the school schedule
- Social anxiety disorder: she is not afraid of peers' judgment
- Generalized anxiety disorder: worry is limited to separation and has lasted weeks, not 6 months
- Posttraumatic stress disorder: a parent's appendectomy is not a qualifying traumatic event, and there are no trauma symptoms
- Adjustment disorder: full criteria for separation anxiety disorder are met

Checkpoint: What would you ask or check NEXT?

A. Ask Zainab, in a child-friendly way, what she worries about while she is at school **(best)**

B. Begin a gradual exposure plan for speaking up in class

C. Recommend half days at school until the stomachaches stop

The content of the fear decides between social and separation anxiety, and the child herself is the best source. B treats a social fear that has not been confirmed. C rewards avoidance; keeping the child in school is a core goal of treatment.

The case in six clues (as the app reveals it)

1. Nurse: She's in my office with a stomachache almost every morning, asking me to call home. She has missed nine school days this term.
2. Father: Her pediatrician checked her two weeks ago and found nothing wrong. The stomachaches come on school mornings and Sunday nights, never on Saturdays.
3. Nurse: In class she has gone quiet. She used to chatter with her table, and now she barely talks to classmates and keeps watching the door.
4. Zainab: The kids are nice. I just keep thinking something bad will happen to Daddy while I'm at school, like a car crash, and then nobody would come get me.
5. Father: At home she follows me from room to room and won't sleep in her own bed. She cried until I picked her up from her cousin's sleepover, and she has nightmares about getting lost at the mall.
6. Father: It started about seven weeks ago, after I was in the hospital for three days to have my appendix out. Before that she slept alone and loved school, and her worries are only about me and her mom.

What a skilled clinician weighs after each clue

1. Separation anxiety disorder, Social anxiety disorder, Somatic symptom disorder, Generalized anxiety disorder
2. Separation anxiety disorder, Social anxiety disorder, Generalized anxiety disorder
3. Social anxiety disorder, Separation anxiety disorder, Generalized anxiety disorder
4. Separation anxiety disorder, Generalized anxiety disorder, Posttraumatic stress disorder
5. Separation anxiety disorder, Generalized anxiety disorder
6. Separation anxiety disorder

Sources: DSM-5-TR (APA, 2022), Anxiety Disorders: Separation Anxiety Disorder; Walter et al. (2020), AACAP clinical practice guideline for pediatric anxiety disorders, J Am Acad Child Adolesc Psychiatry 59(10):1107-1124

Patient 52: The silent first grader

Setting: Noor, 6, in first grade, seen by the school social worker with her mother and her teacher. Noor sits on her mother's lap and does not speak.

> *"Mother: Everyone says she is shy and she will grow out of it. But she was like this last year too, and now she won't even ask to use the bathroom."*

If asked about…

When did this start?	At preschool last year she never talked to the teachers. We thought it was the first year. This year, three months now, not one word in class.
Speech at home	She talks all day. Arabic with me and her grandmother, English with her brother. She bosses him around in English and sings every song from her cartoons.
Speech outside home	At the park with her cousins she is loud and silly. At the doctor's office or with a new grown-up, nothing. She whispers to me if no one is close. The school has an aide who speaks Arabic, and Noor won't talk to her either.
Language background	She was born here. Her brother is in fourth grade and speaks only English with her. She went to an English-speaking preschool for a year.
Sleep	She sleeps well, ten hours. Some nights before school she says her tummy hurts.
Medical history and medications	Her hearing test was normal at her checkup. The speech therapist watched a video of her talking at home and said her speech is right for her age in both languages. No medicines. She walked and talked on time, early even.
Safety and stressful events	No one has hurt her, and nothing scary happened. She is a happy girl at home.
Family history	I was very shy as a child. My husband's sister did not speak at school until second grade, and she is a teacher now.
School functioning	Her work is good. But she wet her pants last week because she would not ask to go. The other children speak for her. At the fall show she danced in front of the whole school and loved it. She just won't talk.
Culture and context	We came from Jordan before she was born. My mother says children are often quiet at first in a new place, so don't worry. But Noor was born here and knows English, and the teacher cannot hear her read. I worry.

Only if asked (don’t volunteer)

- Speaks fluent English at home with her brother
- Same silence at preschool last year
- Silent with the Arabic-speaking aide too
- Speech therapist found age-typical language in both languages

Patient 52: answer and debrief

DIAGNOSIS

Selective mutism

Silence in a child from an Arabic-speaking home suggested a normal language-learning period. Clue 4 flips it: she speaks English fluently at home, so a lack of English cannot explain her silence. She also stays silent with an Arabic-speaking aide, and the silence has lasted about three months, well past the first month of school, after the same pattern in preschool. Exam lesson: assess language knowledge and comfort first (a recent immigrant learning English does not have selective mutism), check hearing and language, and treat with gradual behavioral steps shared by school and family rather than pressure to speak.

What had to be ruled out

- Normal second-language silent period: she speaks English fluently at home and is silent in Arabic at school too
- Hearing loss or a language disorder: normal hearing test; speech therapist found age-typical language in both languages
- Autism spectrum disorder: she gestures, shares attention, plays with a friend and is playful with family
- Social anxiety disorder: she does not avoid being watched or performing; the problem is speaking
- Trauma-related silence: no traumatic event before onset

Checkpoint: What would you ask or check NEXT?

A. Give her more time to learn English before doing anything

B. Ask the family how and with whom she speaks at home and in which languages, and make sure hearing and language are checked **(best)**

C. Refer her for an autism evaluation

Silence at school in a bilingual child calls first for finding out whether she knows and uses English elsewhere, and whether hearing and language are typical. A assumes a language-learning explanation without checking it. C jumps to autism when she gestures, follows directions and plays with a friend.

The case in six clues (as the app reveals it)

1. Teacher: Noor hasn't said a single word in class since school started about three months ago. I can't hear her read, and she won't even ask to use the bathroom.
2. Teacher: Her family speaks Arabic at home, so I figured she's still picking up English and it would pass.
3. Teacher: She points and nods, follows every direction, and her written work is at grade level. At recess she plays tag with one friend without a word, and when I ask her something directly, she freezes and looks at the floor.
4. Mother: At home she never stops talking. Arabic with me and her grandmother, English with her big brother, and she sings along to English cartoons all day.
5. Mother: It was the same at her English-speaking preschool last year. And the school has an aide who speaks Arabic, and Noor won't speak to her either.
6. Mother: Her hearing test was normal, and the speech therapist watched a video of her at home and said her talking is right for her age in both languages. She danced in front of the whole school at the fall show, beaming; she just won't talk there.

What a skilled clinician weighs after each clue

1. Selective mutism, No mental disorder (normal or expected reaction), Autism spectrum disorder, Social anxiety disorder
2. No mental disorder (normal or expected reaction), Selective mutism, Social anxiety disorder
3. No mental disorder (normal or expected reaction), Selective mutism, Social anxiety disorder
4. Selective mutism, Social anxiety disorder
5. Selective mutism, Social anxiety disorder
6. Selective mutism

Sources: DSM-5-TR (APA, 2022), Anxiety Disorders: Selective Mutism

Patient 53: The boy who doesn't reach

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

Setting: Elijah, 3 and a half, in his fourth month with a foster family, at a foster care agency's early childhood clinic. His foster mother and his county caseworker came with him, and an early intervention evaluator's note is in the file.

> *"Foster mother: I've fostered eleven kids. Most of them run to me by the second week. Four months in, this one still won't let me hold him."*

If asked about…

Placement history	He came into care at two after he was found alone in an apartment for days. He's been in three homes in a year. He's been with me four months.
Comfort	When he's hurt, he sits alone. If I pick him up, he goes stiff and turns his face away. He doesn't come to me when he's scared either.
Play and emotions	He lines up cars, but he'll let me change them. He hardly smiles, even at bubbles. Sometimes in the bath, out of nowhere, he looks terrified or starts crying.
Speech and development	A few words, mostly pointing. He points at a bird so I'll look too, and he likes clapping games. The early intervention lady says his thinking is close to his age; speech is behind. He's starting speech therapy.
Sleep	He falls asleep fine and wakes once a night. No nightmares that I can tell.
Eating	He eats fast, like someone might take it. I find crackers hidden in his pillowcase.
Medical history	He was underweight when he came into care; now he's gaining. Hearing test normal. Lead test normal. No medicines.
Strangers	He's never gone off with anyone. When company comes, he hides behind the couch until they leave.
Safety	He doesn't hurt himself or other kids. No head banging. He's a quiet, careful little boy.
Family history	The caseworker says his birth mother was very depressed and alone. We don't know much about his father.
Visits and context	He has a supervised visit with his birth mother once a month. He doesn't cry when she leaves, and he doesn't cry when I pick him up either. The plan right now is reunification if she completes services.

Only if asked (don’t volunteer)

- Found alone for days with little food before entering care at two
- Three homes in one year
- Hides from visitors; never goes off with strangers
- Lets others move his cars without distress

Patient 53: answer and debrief

DIAGNOSIS

Reactive attachment disorder

Lining up toys and little speech made autism the lead. Clue 3 flips it: severe neglect and repeated changes of caregiver. The evaluator finds shared attention and no restricted, repetitive pattern, so autism criteria are not met. He rarely seeks or accepts comfort, shows little positive emotion and unexplained fear, the pattern began before age 5, and his developmental age is well over 9 months. He is withdrawn rather than overly friendly, which rules out disinhibited social engagement disorder. Exam lesson: treatment centers on a stable, sensitive, responsive caregiver, supported by caregiver-focused interventions; coercive 'holding' or restraint therapies are unsafe and not supported.

What had to be ruled out

- Autism spectrum disorder: shares attention and joins games; no restricted, repetitive pattern
- Disinhibited social engagement disorder: he is withdrawn and wary of strangers, not overly familiar
- Posttraumatic stress disorder: no nightmares, trauma play or other intrusion symptoms
- Intellectual developmental disorder: thinking skills near age level
- Hearing loss or a medical cause: normal hearing; growth improving with adequate food

Checkpoint: What would you ask or check NEXT?

A. Refer him for an autism evaluation and start a behavior program right away

B. Advise the foster mother to give him space until he comes to her on his own

C. Ask the caseworker about his early care, his placements and who has cared for him **(best)**

For a young child in foster care, the history of care is essential before deciding between autism and an attachment disorder. A acts before that history is known. B is harmful: a withdrawn child needs a caregiver who offers comfort warmly and consistently, not one who waits.

The case in six clues (as the app reveals it)

1. Foster mother: When he falls and scrapes his knee, he doesn't cry or come to me. He just sits there by himself.
2. Foster mother: He barely talks and rarely smiles. He will line up his toy cars for an hour on his own.
3. Caseworker: He came into care at two, after he was found alone in an apartment for days with almost no food. This is his third home in a year.
4. Evaluator's note: He points to show me pictures, follows my gaze, and joined a clapping game. When I moved his cars out of line, he did not mind. No rocking, flapping or unusual reactions to sounds or textures; his thinking skills are near his age, his hearing is normal, and his speech is behind.
5. Foster mother: When I pick him up to comfort him, he goes stiff and looks away. Sometimes, in the middle of a happy bath, he suddenly cries or looks scared, and I can't see why.
6. Foster mother: He has never wandered off with a stranger; when visitors come, he hides behind the couch. He doesn't have nightmares or play out scary scenes.

What a skilled clinician weighs after each clue

1. Reactive attachment disorder, Autism spectrum disorder, Posttraumatic stress disorder
2. Autism spectrum disorder, Reactive attachment disorder, Intellectual developmental disorder
3. Reactive attachment disorder, Autism spectrum disorder, Disinhibited social engagement disorder
4. Reactive attachment disorder, Disinhibited social engagement disorder, Posttraumatic stress disorder
5. Reactive attachment disorder, Posttraumatic stress disorder
6. Reactive attachment disorder

Sources: DSM-5-TR (APA, 2022), Trauma- and Stressor-Related Disorders: Reactive Attachment Disorder; Chaffin et al. (2006), Report of the APSAC Task Force on Attachment Therapy, Reactive Attachment Disorder, and Attachment Problems, Child Maltreatment 11(1):76-89

Patient 54: The lunch money

Setting: Caleb, 13, in eighth grade, at a community mental health clinic after his second suspension. His mother came with him, and the school counselor sent a note.

> *"Caleb: My mom's making a big deal out of nothing. Can we get this over with?"*

If asked about…

When did this start?	Mother: Around his twelfth birthday, about a year and a half ago. Before that he was easy. Good grades, Scouts, soccer.
Fights and threats	Caleb: I've never actually hit anybody. I just tell them I will. They give me the money, so I don't have to.
Stealing and property	Caleb: The car thing, yeah. It was unlocked. Mother: He took a speaker. The neighbor didn't press charges, but he could have.
Staying out	Mother: Two or three nights a month he doesn't come home. I call the police. He shows up the next morning like nothing happened. It started when he was twelve.
Animals and fire	Mother: No, he's good with our dog. Never set fires that I know of.
Mood	Mother: He's not an angry kid. He's calm, even charming, until he doesn't get his way. He's not spiteful with me or his sister.
Sleep	Mother: When he's home he sleeps fine. Up late on his phone, like any kid.
Alcohol, drugs, medications	Caleb: I don't do drugs. The doctor tested me. I tried vaping once. No medications.
Medical history	Mother: Healthy. No head injuries. Hearing and vision fine.
Safety	Caleb: I don't want to hurt myself. That's dumb. Mother: There are no guns or weapons in our house.
Family history	Mother: His father had trouble with the law when he was young. We divorced when Caleb was five; he sees him summers.
Friends and context	Mother: When he was twelve he started hanging out with older kids from the skate park. I work double shifts at the plant, so he's home alone a lot after school.

Only if asked (don’t volunteer)

- Taking younger students' lunch money with threats for about two months
- Broke into a neighbor's car about seven months ago
- Stays out all night two or three times a month since age 12
- Not angry, touchy or spiteful at home

Patient 54: answer and debrief

DIAGNOSIS

Conduct disorder

Talking back and refusing rules looked like oppositional defiant disorder. Clue 4 flips it: threatening younger students and taking their money violates others' rights. With a car break-in and staying out all night since 12, he shows 3 or more conduct behaviors in the past 12 months, at least one in the past 6: conduct disorder, adolescent-onset type, since nothing began before age 10. Exam lesson: ODD is angry, defiant or spiteful behavior; CD is violating others' rights or major age rules. Both can be diagnosed when both are fully met, but he lacks the angry or spiteful mood ODD needs. The school must also act to protect the younger students.

What had to be ruled out

- Oppositional defiant disorder: fewer than 4 symptoms; he is not angry, touchy or spiteful (both may be given when both are fully met)
- Disruptive mood dysregulation disorder: no persistent irritable mood; onset after age 10
- ADHD: no attention problems or hyperactivity; good grades before age 12
- Substance use: clean drug test last month
- Antisocial personality disorder: requires age 18 or older

Checkpoint: What would you ask or check NEXT?

A. Ask about aggression, threats, stealing, property damage and staying out, and get reports from the school **(best)**

B. Start parent management training focused on defiance at home

C. Reassure his mother that defiance at this age is common and usually passes

Defiance alone does not tell you whether he is violating others' rights, which separates oppositional defiant disorder from conduct disorder and changes risk and treatment. B treats before the behavior is fully assessed. C dismisses a pattern that has led to two suspensions and may hide harm to others.

The case in six clues (as the app reveals it)

1. Mother: He's been suspended twice this year. He talks back to teachers, ignores every rule at home, and tells me 'you can't make me.'
2. Caleb: Teachers are on me for nothing. I'm not mad all the time; I just don't like being told what to do. And I don't do drugs; my doctor tested me last month and it was clean.
3. Mother: It started about a year and a half ago, around his twelfth birthday; before that he was an easy kid with good grades. He's not moody or spiteful at home. He's pleasant until someone tells him no.
4. School counselor's note: Two sixth graders report that for about two months Caleb has been taking their lunch money and telling them he'll beat them up if they tell.
5. Mother: About seven months ago he broke into a neighbor's car and took a speaker, and he admitted it. Since he was twelve he has stayed out all night two or three times a month, even though I've called the police.
6. Caleb: Those kids are babies, and the guy should've locked his car. I don't feel bad about it.

What a skilled clinician weighs after each clue

1. Oppositional defiant disorder, ADHD, Conduct disorder, Disruptive mood dysregulation disorder
2. Oppositional defiant disorder, Conduct disorder, ADHD
3. Oppositional defiant disorder, Conduct disorder
4. Conduct disorder, Oppositional defiant disorder
5. Conduct disorder, Oppositional defiant disorder
6. Conduct disorder

Sources: DSM-5-TR (APA, 2022), Disruptive, Impulse-Control, and Conduct Disorders: Conduct Disorder; DSM-5-TR (APA, 2022), Disruptive, Impulse-Control, and Conduct Disorders: Oppositional Defiant Disorder

Patient 55: The broken phone

Setting: Rebecca, 34, a paralegal, referred to the behavioral health clinician in her primary care office. Her husband came with her, and her doctor's note is in the chart.

> *"My doctor asked about stress and I started crying. I'm not a bad person. I just explode, and then I hate myself."*

If asked about…

When did this start?	It's been building for years, but the last year and a half has been the worst. Two or three times a week now.
What an outburst is like	Something small happens, like a slow driver or a printer jam, and I'm screaming before I know it. Twenty minutes later it's over and I'm shaking and embarrassed.
Damage and violence	This past year I broke my phone, kicked a hole in the bedroom door and broke a mug. I've never hit or threatened a person or an animal. My husband says he isn't afraid of me, but he's tired.
Mood between outbursts	Normal. I like my job, I like my life. I'm not sad or irritable between them.
Sleep	Seven hours. I've never gone days without sleep feeling amazing.
Alcohol, drugs, medications	I used to drink on weekends, sometimes too much. I quit four months ago. The blowups didn't stop. No drugs. Only birth control.
Medical history	My doctor checked everything, thyroid too. No concussions, no seizures.
Safety	No thoughts of hurting myself. No guns in the house. I'd never hurt anyone on purpose.
Family history	My dad had a temper like this. He'd yell and slam doors, and then act like nothing happened.
Work and relationships	My boss gave me a final warning after I yelled at a copier repairman. We've been married nine years and it's solid, but he says he's walking on eggshells.
Culture and context	In my family, anger was the only feeling anyone showed. I swore I wouldn't be like my dad.

Only if asked (don’t volunteer)

- Sober for four months and outbursts continued
- Outbursts two or three times a week for well over a year
- Final warning at work
- Never hit or threatened a person; no firearms at home

Patient 55: answer and debrief

DIAGNOSIS

Intermittent explosive disorder

The worst outbursts happened while drinking, so alcohol was the lead. Clue 3 flips it: the outbursts continued through four sober months. She has two or three verbal outbursts a week (the threshold is about twice a week for 3 months, so the sober months alone qualify), all impulsive, out of proportion and followed by shame, with normal mood between: intermittent explosive disorder. Exam lesson: normal mood between outbursts separates IED from DMDD and bipolar; remorse and the lack of planning or gain separate it from antisocial patterns; rule out substances, head injury and medical causes, and assess violence risk, including toward a partner.

What had to be ruled out

- Alcohol-related aggression: outbursts continued through four months of sobriety
- Bipolar I disorder: no episodes of elevated mood or reduced need for sleep
- Borderline personality disorder: stable relationships, no fear of abandonment or emptiness
- Personality change due to another medical condition: no head injury or seizures; normal exam and thyroid
- Posttraumatic stress disorder: no trauma history
- Antisocial personality disorder: outbursts are unplanned, bring no gain and are followed by remorse

Checkpoint: What would you ask or check NEXT?

A. Advise her to stop drinking and see whether the outbursts stop

B. Ask whether the outbursts also happen when she has not been drinking, how often they occur, and about her drinking pattern **(best)**

C. Refer her to an anger management group

Whether the outbursts happen only with alcohol decides between a substance problem and an independent disorder, so assess that before intervening. A gives advice before the pattern is known; she may already have stopped. C is an intervention chosen before the cause is clear.

The case in six clues (as the app reveals it)

1. I lose it over small things. Last week I screamed at a cashier because she bagged my groceries wrong, and afterward I sat in my car shaking.
2. Husband: The worst ones this past year came after she'd been drinking. She smashed her phone at my brother's wedding and kicked a hole in our bedroom door after a night out.
3. Husband: She quit drinking completely four months ago because of that, and the blowups kept coming: at seven in the morning in traffic, at her desk, at home. Last month, sober, she threw a mug against the kitchen wall and broke it.
4. It's two or three yelling blowups a week, for well over a year. Each one comes out of nowhere, lasts maybe twenty minutes, and then I'm myself again, just ashamed. I never plan it, and I don't get anything out of it.
5. My mood is fine in between. I've never had days of feeling on top of the world without needing sleep. We've been married nine years; I don't panic about people leaving, and I don't feel empty.
6. Doctor's note: Normal physical exam and thyroid tests. No head injury, seizures or trauma history, and no medications except birth control.

What a skilled clinician weighs after each clue

1. Intermittent explosive disorder, Borderline personality disorder, Bipolar I disorder, Major depressive disorder
2. Alcohol use disorder, Intermittent explosive disorder, Borderline personality disorder
3. Intermittent explosive disorder, Borderline personality disorder, Bipolar I disorder
4. Intermittent explosive disorder, Borderline personality disorder, Bipolar I disorder
5. Intermittent explosive disorder, Posttraumatic stress disorder
6. Intermittent explosive disorder

Sources: DSM-5-TR (APA, 2022), Disruptive, Impulse-Control, and Conduct Disorders: Intermittent Explosive Disorder

Patient 56: The court date

Setting: Kyle, 26, a roofer, at a county jail mental health intake two weeks after booking. A jail nurse and a housing-unit officer add notes.

> *"I need to go to the hospital. The voices are really bad. I can't be in here."*

If asked about…

The voices	They talk all the time, every second, in rhymes. They say kill yourself. (If asked what else they say, he gets vague: 'Just stuff. Bad stuff.')
When did this start?	Right after I got locked up. Maybe the day after I talked to my lawyer.
The night of the arrest	I don't remember any of it. I'd had a couple of beers. I wasn't that drunk.
Sleep and appetite	I sleep okay, I guess. The food's bad, but I eat it.
Alcohol, drugs, medications	Beers on weekends. A little weed sometimes, not lately. No medications.
Medical history	Never been sick. Fell off a roof once and broke my wrist, didn't hit my head.
Psychiatric history	Never saw a counselor or anything. First time this has happened.
Safety	No, I'm not going to do anything. I've never tried to hurt myself. I just need to get out of here and go to the hospital.
Family history	My uncle has schizophrenia. I saw how he was. He went to the state hospital a lot.
Legal situation	Burglary. My public defender says I could get a few years. Trial's in three weeks. Do people who hear voices still have to go to court?
Daily life on the unit	I play cards. It passes the time. The voices are worse when I'm alone.
Culture and context	I've been roofing since I was seventeen. I have a daughter, she's four. I can't do a few years. I can't.

Only if asked (don’t volunteer)

- Voices began the day after his trial date was set
- Has asked staff whether voices would get him sent to the hospital instead of court
- Plays cards and talks clearly on the unit when staff are not watching
- Uncle with schizophrenia who was hospitalized often

Patient 56: answer and debrief

DIAGNOSIS

Malingering

Unexplained voices with a clean screen first suggested a primary psychotic disorder, though his description was unusual: nonstop, rhyming, with no other signs of psychosis. Clue 4 shows a gap between what he reports and how he functions when unobserved, and clue 5 flips it: the symptoms began when his trial was set, with a stated goal of avoiding court. Faked symptoms for an external reward are malingering, which DSM-5-TR lists as a condition that may be a focus of clinical attention, not a mental disorder; factitious disorder has no external reward. Exam lesson: suspect malingering in legal settings with discrepant findings, document observations objectively, do not accuse, and never skip a suicide risk assessment because you suspect faking.

What had to be ruled out

- Substance/medication-induced psychotic disorder: clean booking screen; no withdrawal signs
- Brief psychotic disorder or another primary psychosis: organized speech and behavior when unobserved; no confusion, negative symptoms or sleep change
- Psychotic disorder due to another medical condition: normal vitals and labs
- Factitious disorder imposed on self: there is a clear external incentive (avoiding trial)

Checkpoint: What would you ask or check NEXT?

A. Complete a suicide risk assessment and follow the jail's safety protocol, then gather records and staff observations **(best)**

B. Tell him the voices he describes don't sound real

C. Request his transfer to the state hospital for treatment of psychosis

Any report of command voices to die must be assessed for safety first, whatever you suspect, and jail settings carry raised suicide risk. B is confrontational, damages rapport and decides before the facts are in. C acts on his report alone, before medical, substance and collateral information is gathered.

The case in six clues (as the app reveals it)

1. I hear voices day and night, and I can't remember anything about the night they arrested me.
2. The voices tell me to kill myself. They never stop, not for one second, and they talk in rhymes like a song.
3. Nurse: His booking drug screen was clean, and his vitals and labs are normal. We can find no psychiatric history in any records, and he was not confused at booking.
4. Officer: On the unit he plays cards for hours, laughs and argues sports clearly. When staff walk by, he starts holding his head and mumbling.
5. Nurse: The voices began the day after his lawyer told him his trial date was set. Twice he has asked me whether hearing voices will get him sent to the state hospital instead of going to court.
6. Nurse: He sleeps eight hours and eats every meal. In my safety check he said he has no plan or intent to hurt himself and has never tried; he says he wants 'the hospital, not court.'

What a skilled clinician weighs after each clue

1. Substance/medication-induced psychotic disorder, Brief psychotic disorder, Schizophrenia, Malingering
2. Brief psychotic disorder, Substance/medication-induced psychotic disorder, Malingering
3. Brief psychotic disorder, Malingering, Factitious disorder imposed on self
4. Malingering, Brief psychotic disorder, Factitious disorder imposed on self
5. Malingering, Factitious disorder imposed on self
6. Malingering

Sources: DSM-5-TR (APA, 2022), Other Conditions That May Be a Focus of Clinical Attention: Malingering; DSM-5-TR (APA, 2022), Somatic Symptom and Related Disorders: Factitious Disorder

Patient 57: The bus ticket

Setting: Hamid, 46, who runs his family's print shop, in a hospital emergency department, seen by the ED social worker. The ED physician adds notes, and his younger brother joins by phone.

> *"Please, tell me what day it is. My brother says I've been gone almost a week. That's not possible."*

If asked about…

What do you remember?	Sitting in my car outside the shop. Then a police officer at a bus station asking if I was okay. Nothing in between. Before the parking lot I remember everything.
The trip	They say I bought a ticket myself and slept in a motel under my own name. I don't remember any of it.
The weeks before	My wife told me she's leaving. The same week we moved my mother into a nursing home. I kept working. I told everyone I was fine.
Sleep	Before this, maybe four hours a night for two weeks. I never take anything for sleep.
Alcohol, drugs, medications	I don't drink. No drugs. One blood pressure pill in the morning, for years.
Medical history	High blood pressure. No seizures, no fainting, no head injuries. Never lost time before.
Other memory gaps or identities	No. Nobody has ever called me by another name or told me I did things I don't remember. I've never found things I don't remember buying.
Safety	The week before, sitting in the car, I wondered if anyone would miss me. I never made a plan and I never tried anything. Right now I just want to go home. There are no guns at my house.
Family history	My father had depression after he left Iran. No one in the family has had anything like this.
Work and functioning	I've run the shop for fifteen years. Six employees. I've never missed a day until this.
Culture and context	I'm the oldest son. Everyone leans on me. I don't tell people when I'm struggling.

Only if asked (don’t volunteer)

- Wife announced she was leaving and mother moved to a nursing home the week before
- No sleeping pills, seizures or earlier gaps in memory
- Passive thought the week before about whether anyone would miss him; no plan, intent or attempt
- No court case, debts or insurance claim

Patient 57: answer and debrief

DIAGNOSIS

Dissociative amnesia

Lost time with travel first suggested intoxication, a head injury or delirium. Clue 3 flips it: normal alcohol and drug screens, a normal CT and neurological exam, and clear attention. He cannot recall five days after major stressors, during which he traveled on purpose: dissociative amnesia with dissociative fugue. No sleeping pills, seizures, other identities or outside gain remain. Exam lesson: rule out substances (including sedative sleep medicines) and medical causes first; then screen for DID, PTSD and malingering. Assess suicide risk, which can rise as painful memories return.

What had to be ruled out

- Alcohol or drug effects, including sedative sleep medicines: zero blood alcohol, negative screen, no sleeping pills
- Head injury: normal CT and neurological exam, no bruises
- Seizures or transient global amnesia: no seizure history; transient global amnesia lasts hours, not days
- Delirium: alert, oriented, attention intact
- Dissociative identity disorder: no other identities and no earlier gaps in memory
- Posttraumatic or acute stress disorder: the stressors, though severe, are not traumatic events of the kind these disorders require
- Malingering: no legal, financial or other external gain

Checkpoint: What would you ask or check NEXT?

A. Make sure he has a medical workup, including an alcohol and drug screen, head imaging and a neurological exam **(best)**

B. Begin trauma-focused therapy to help him recover the lost days

C. Call his family to fill in the gap and plan discharge home

Sudden memory loss in a man in his 40s calls for ruling out substances, head injury, seizures and delirium first. B is premature; pushing to recover memories is not a first step. C skips the medical and safety assessment.

The case in six clues (as the app reveals it)

1. I don't know how I got here. I remember sitting in my car outside my shop, and then a police officer talking to me at a bus station. Everything in between is blank.
2. ED physician: Transit police found him at a bus station about 300 miles from home. His family reported him missing five days ago, and a receipt in his pocket shows he bought the bus ticket himself.
3. ED physician: Blood alcohol is zero, the drug screen is negative, and his head CT and neurological exam are normal, with no bruises. He is alert, knows the date and place, and his attention is sharp.
4. Brother: The week before he vanished, his wife told him she was leaving, and we moved our mother into a nursing home. He kept saying he was fine.
5. I take only a blood pressure pill, and I've never taken sleeping pills. I've never had a seizure or lost time before, and nobody has ever told me I acted like a different person.
6. Brother: He has no court case, no debts he's running from and no insurance claim. All he keeps asking is when he can get back to the shop.

What a skilled clinician weighs after each clue

1. Delirium, Alcohol intoxication, Dissociative amnesia
2. Delirium, Dissociative amnesia, Alcohol intoxication
3. Dissociative amnesia, Malingering, Dissociative identity disorder
4. Dissociative amnesia, Malingering, Dissociative identity disorder
5. Dissociative amnesia, Malingering
6. Dissociative amnesia

Sources: DSM-5-TR (APA, 2022), Dissociative Disorders: Dissociative Amnesia

Patient 58: The flat dorm room

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

Setting: Riley, 21, an architecture student who uses they/them pronouns, at a university counseling center by telehealth. A note from the campus health physician is in the chart.

> *"This is going to sound weird. I'm not crazy. I just feel like I'm not really here."*

If asked about…

When did this start?	Eight months ago, at a party. I ate a gummy, panicked, thought I was dying. The panic ended. The fog never did.
What it feels like	Like I'm a camera floating behind my head. My hands look like someone else's. My room looks like a set. It's every day, all day.
Do you think it's real?	No. I know I'm real and the world is real. It just doesn't feel real. That's what makes it so scary.
Panic attacks	Just that one night. Since then I get anxious about the fog, but no more attacks. The fog is the same whether I'm calm or freaking out.
Sleep	I sleep fine, seven or eight hours. Waking up is the worst part, realizing it's still here.
Alcohol, drugs, medications	Nothing since that night. Not a drop of alcohol, no weed, nothing. No medications.
Medical history	Campus health did bloodwork and a neuro exam. All normal. No seizures, no concussions.
Mood and safety	I'm anxious, not depressed. I still like music and my friends. No thoughts of killing myself. I just want to feel like me again.
Trauma	Nothing like that. My childhood was okay. Nothing bad happened at the party except the gummy.
Family history	My mom has anxiety. That's all I know of.
School	I dropped a studio class. I can't feel connected to anything I design, so I can't decide anything.
Culture and context	I'm the first in my family to go to college. Everyone's proud. I haven't told them about this.

Only if asked (don’t volunteer)

- No substance use of any kind for eight months
- No panic attacks since the first night; the feeling is constant
- Knows the world is real (reality testing intact)
- Normal neurological exam and labs

Patient 58: answer and debrief

DIAGNOSIS

Depersonalization/derealization disorder

Onset during a cannabis-triggered panic attack made a substance effect or panic disorder the lead. Clue 4 flips it: eight months without any use, no further panic attacks, and the unreality is constant. Medical causes such as seizures are ruled out, and Riley knows the feeling is not real, so reality testing is intact, which separates this from psychosis. Exam lesson: drugs, including cannabis, can trigger depersonalization; when it persists long after the substance is gone, the diagnosis is depersonalization/derealization disorder. Unreality that occurs only during panic attacks, intoxication or PTSD is not diagnosed separately.

What had to be ruled out

- Substance effect: no cannabis or other use for eight months, yet symptoms persist
- Panic disorder: one attack, during intoxication; the unreality is constant, not limited to attacks
- Schizophrenia or another psychotic disorder: reality testing intact; no hallucinations or delusions
- Medical cause such as seizures: normal neurological exam and labs
- Major depressive disorder: mood anxious but not depressed; sleep and appetite normal
- Posttraumatic stress disorder: no traumatic event

Checkpoint: What would you ask or check NEXT?

A. Reassure them that this is a leftover cannabis effect that will wear off on its own

B. Refer them for an antipsychotic medication evaluation

C. Ask about any substance use since that night, and whether the unreal feeling comes only during panic attacks or all the time **(best)**

Whether the symptom depends on ongoing use or on panic attacks decides between a substance effect, panic disorder and a dissociative disorder. A assumes a cause without checking and may leave a treatable disorder unaddressed. B assumes psychosis before reality testing has been assessed.

The case in six clues (as the app reveals it)

1. For months everything has felt off, like I'm watching my life through a window.
2. My hands don't look like mine, and my voice sounds like a recording. My dorm room looks flat, like a movie set.
3. It started eight months ago at a party, after I ate a cannabis gummy. My heart pounded and I thought I was dying, and when the panic faded, this feeling stayed.
4. I haven't used weed or anything else since that night, and I haven't had another panic attack. The feeling is there all day, every day, whether I'm calm or anxious.
5. Physician's note: Normal neurological exam, bloodwork and thyroid. No seizures, head injuries or medications; mood is anxious but not depressed, and sleep and appetite are normal.
6. I know the world is real and my hands are mine; it just doesn't feel that way, and that's what scares me. I dropped a studio class because I can't connect to anything I design.

What a skilled clinician weighs after each clue

1. Depersonalization/derealization disorder, Major depressive disorder, Posttraumatic stress disorder
2. Depersonalization/derealization disorder, Schizophrenia, Substance/medication-induced psychotic disorder
3. Substance/medication-induced psychotic disorder, Panic disorder, Depersonalization/derealization disorder
4. Depersonalization/derealization disorder, Major depressive disorder, Schizophrenia
5. Depersonalization/derealization disorder, Schizophrenia
6. Depersonalization/derealization disorder

Sources: DSM-5-TR (APA, 2022), Dissociative Disorders: Depersonalization/Derealization Disorder

Patient 59: The mirror hours

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

Setting: Rohan, 31, a hotel front-desk supervisor, at an outpatient clinic on his dermatologist's referral. His partner, Lucas, came with him, and the dermatologist's note is in the chart.

> *"My dermatologist sent me. She thinks it's in my head. If you could see my skin under good light, you'd understand."*

If asked about…

When did this start?	In high school I worried about acne. The last four years it's taken over. The last year I've stopped going out.
Mirror and checking	About three hours a day. Every mirror, every car window. I check my phone camera before I walk into work.
Picking	I pick at bumps to flatten them, only on my face. Never when I'm bored. Only to fix it. Then it's red and I feel worse.
Treatments	Two laser treatments, three dermatologists. Each time I thought it looked worse. I'm saving for a third.
How others see it	Lucas says he can't see anything. My sister says the same. I think they're being kind.
Sleep	I'm up late in the bathroom mirror. Maybe six hours a night.
Alcohol, drugs, medications	A beer or two on weekends at home. No drugs. A topical cream from the dermatologist.
Medical history	Healthy. The dermatologist says my skin is normal.
Mood and safety	Low a lot. Some nights I think I'd rather be dead than look like this. I've never made a plan or tried, and I wouldn't do that to Lucas. No guns at home.
Family history	My mom checks the stove a lot. My uncle had depression.
Work and social life	I switched to back-office shifts. We used to go out every weekend. Now I cancel.
Culture and context	My family always commented on everyone's looks: who gained weight, whose skin was bad. Growing up gay, I also felt everyone was watching me. Now I can't stop watching myself.

Only if asked (don’t volunteer)

- Picking is only to fix how his face looks
- Passive wish to be dead some nights; no plan, intent or attempt
- Saving for a third laser treatment
- No concern about weight or body shape

Patient 59: answer and debrief

DIAGNOSIS

Body dysmorphic disorder

Hours at the mirror and picking made excoriation disorder the lead. Clue 3 flips it: the 'flaws' are slight to others. Clue 5 adds that the picking aims only to fix how he looks, so it is part of body dysmorphic disorder rather than a separate diagnosis. Checking, reassurance seeking and comparing are the repetitive behaviors; insight is fair, and the concern is not weight or body fat. Exam lesson: cosmetic treatment rarely helps and can worsen BDD; suicide risk is high, so always ask; CBT and SSRIs are first-line.

What had to be ruled out

- Excoriation disorder: picking is done only to fix a perceived flaw in appearance
- Delusional disorder: he has fair insight; even absent insight would stay BDD
- Eating disorder: no concern with weight or body fat
- Obsessive-compulsive disorder: preoccupation is limited to appearance
- Social anxiety disorder: avoidance is driven by the perceived flaw, not fear of social judgment in general

Checkpoint: What would you ask or check NEXT?

A. Refer him for habit reversal training to stop the skin picking

B. Ask what the picking and mirror checking are meant to do, how others see his skin, and about his mood and any thoughts of suicide **(best)**

C. Encourage him to ask a cosmetic dermatologist about a stronger laser treatment

The purpose of the picking and how real the flaw is decide between excoriation disorder and body dysmorphic disorder, and appearance preoccupation carries high suicide risk, so ask. A treats the picking before its purpose is known. C feeds the preoccupation; cosmetic procedures rarely help and often worsen distress.

The case in six clues (as the app reveals it)

1. I've stopped going out on weekends, and I work in the back office whenever I can. I don't want people looking at me.
2. It's my skin. My cheeks are covered in pits and marks. I spend about three hours a day at the mirror, and I pick at bumps to smooth them out.
3. Dermatologist's note: His skin is essentially normal: a few faint marks, smaller than freckles, that most people would never notice. The redness he worries about comes from the picking.
4. Lucas: He asks me twenty times a day whether his skin looks bad, and he compares himself to every man on TV. He's had two laser treatments, and after each one he said it looked worse.
5. I only pick at my face, and only to fix how it looks. I don't do it when I'm bored or stressed, and if my skin looked normal I'd stop tomorrow.
6. Part of me knows it's probably not as bad as I think. I don't care about my weight or my body; it's just my face.

What a skilled clinician weighs after each clue

1. Social anxiety disorder, Major depressive disorder, Body dysmorphic disorder, Avoidant personality disorder
2. Excoriation disorder, Body dysmorphic disorder, Obsessive-compulsive disorder
3. Body dysmorphic disorder, Excoriation disorder, Delusional disorder
4. Body dysmorphic disorder, Excoriation disorder
5. Body dysmorphic disorder, Delusional disorder
6. Body dysmorphic disorder

Sources: DSM-5-TR (APA, 2022), Obsessive-Compulsive and Related Disorders: Body Dysmorphic Disorder; DSM-5-TR (APA, 2022), Obsessive-Compulsive and Related Disorders: Excoriation (Skin-Picking) Disorder

Patient 60: The 3 a.m. clock

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

Setting: Tariq, 66, a retired postal carrier, referred to the behavioral health clinician in his primary care office. His wife came with him, and his doctor's note is in the chart.

> *"My doctor says I should talk to you about my sleep. I don't know what talking will do. I need sleep."*

If asked about…

When did this start?	About seven months ago, when my wife was in the hospital with pneumonia. She's fine now. My sleep never came back.
A typical night	Bed at ten. Awake until one, or asleep then up at three. I lie there watching the clock. Up for good at six-thirty. Four or five nights a week.
Daytime	Tired, short-tempered. I nap in my chair after lunch sometimes. I stopped volunteering at the food bank because I was too worn out.
Alcohol	About four months ago I started two big glasses of wine at bedtime. It knocked me out, but I'd wake at three anyway. My doctor told me to stop. I stopped nine weeks ago. Sleep is the same.
Other substances and medications	One blood pressure pill, same for six years. One coffee with breakfast. I don't smoke. Nothing for sleep.
Snoring and breathing	My wife says I don't snore. They gave me a test to wear at home, and it was normal.
Medical history	High blood pressure. Knees ache sometimes but don't wake me. No other problems.
Mood and safety	I'm not depressed. I still enjoy golf and my grandkids. No, I've never thought of ending my life.
Worry	Only about sleep. By nine at night I start dreading bed. Otherwise I'm not a worrier.
Family history	My father was a light sleeper. No mental illness I know of.
Culture and context	I carried mail for thirty-four years, up at four every morning. When I retired I thought I'd finally sleep. I pray at dawn, and I used to go right back to sleep. Now I can't.

Only if asked (don’t volunteer)

- Insomnia began about seven months ago, before the wine
- Stopped the wine nine weeks ago with no change
- Normal home sleep apnea test
- Worry limited to sleep; mood and interests intact

Patient 60: answer and debrief

DIAGNOSIS

Insomnia disorder

Poor sleep with irritability first suggested depression, then the bedtime wine suggested an alcohol problem. Clue 4 flips it: the insomnia began months before the wine and did not improve after he stopped. With a normal sleep apnea test, no medication or caffeine cause, no depression, and worry limited to sleep itself, this is insomnia disorder: 4 or 5 nights a week for about 7 months (at least 3 nights a week for 3 months), despite enough time in bed. Exam lesson: CBT for insomnia is first-line before medication; in older adults avoid benzodiazepines and similar sleep drugs when possible because of falls and confusion, and never combine sleeping pills with alcohol.

What had to be ruled out

- Alcohol-related sleep problem: insomnia began before the wine and continued after nine weeks without it
- Sleep apnea: normal home sleep test; no snoring or gasping
- Medication or caffeine: one unchanged blood pressure pill; one morning coffee
- Major depressive disorder: mood and interest intact
- Generalized anxiety disorder: worry is limited to sleep
- Adjustment disorder: insomnia disorder criteria are met, and his wife recovered months ago

Checkpoint: What would you ask or check NEXT?

A. Ask his doctor about a sleeping pill so he can stop the wine

B. Give him a sleep hygiene handout and schedule a follow-up in three months

C. Ask when the sleep problem began compared with the wine, how much he drinks, about snoring or breathing pauses, his medicines and his mood **(best)**

Timing, substances, medical causes such as sleep apnea, and mood must be sorted out before naming or treating the sleep problem. A adds a sedative before assessment, and sleeping pills with alcohol are dangerous, especially in older adults. B delays assessment and offers too little for a long-standing problem.

The case in six clues (as the app reveals it)

1. I can't sleep. I'm exhausted all day, I snap at my wife, and I don't feel like myself.
2. I'm in bed from ten to six-thirty. I lie awake until one, or I wake at three and can't get back to sleep, four or five nights a week.
3. About four months ago I started having two big glasses of wine at bedtime to knock myself out.
4. Wife: His sleep went bad about seven months ago, when I was in the hospital with pneumonia, months before the wine. He stopped the wine nine weeks ago when the doctor asked, and his sleep is no better.
5. Doctor's note: Home sleep apnea test was normal, and his wife reports no snoring or gasping. He takes only a blood pressure pill, unchanged for six years, and one cup of coffee in the morning.
6. I'm not sad. I still enjoy golf with my brother and my grandkids, and my wife is well now. The only thing I worry about is sleep itself; when I lie down, I already know it's going to be a bad night.

What a skilled clinician weighs after each clue

1. Major depressive disorder, Generalized anxiety disorder, Insomnia disorder
2. Insomnia disorder, Major depressive disorder, Generalized anxiety disorder
3. Alcohol use disorder, Insomnia disorder, Major depressive disorder
4. Insomnia disorder, Major depressive disorder, Generalized anxiety disorder
5. Insomnia disorder, Major depressive disorder, Generalized anxiety disorder
6. Insomnia disorder

Sources: DSM-5-TR (APA, 2022), Sleep-Wake Disorders: Insomnia Disorder; Qaseem et al. (2016), ACP guideline on chronic insomnia, Annals of Internal Medicine 165:125-133; American Geriatrics Society Beers Criteria Update Expert Panel (2023), Journal of the American Geriatrics Society 71(7):2052-2081